

patient name - Hari  
age -22 yr old  
sex- prefer not to say  
location : saptarishi clinic  
Reason : routine check up

Subjective :  
patient reports to have severe pain while chewing food or drinking beverages (hot/cold)

denies to have taken any medication causing it.

pain caused when brushing .

irritation when trying to open mouth .

reports pain mostly in the upper jaw .

Objective :  
No acid reflux  
No teeth cavity founded.  
No decaying noticed .  
No inflammation in tongue noticed.  
Inflammation on the upper gums noticed .  
swelling and redness observed in the region .  
No other oral problems noticed .  
Other vitals look normal.  
No plaque was seen on the site .

Assessment:  
Acute Gingivitis

Plan:  
oral checkup was ordered  
anti inflammatory medication prescribed mainly prednisolone.  
taking care on the diet  
to reduce and hot or cold food if possible  
no acidic food allowed on the diet  
another visit if the problem is not resolved .  
might want to be taken care before becoming chronic.

ICD = K05.01  
CPT = 99213 ( low level) due to acute condition of the disease.

Patient name - Krishnan H  
Age - 89  
sex - male  
location - birla hospital  
reason - critical condition checkup

Subjective :  
patient reports severe chest pain and then faints out.

review of system  
non responsive to any contact or waking up.  
shallow breaths  
decreased heart beat  
wheezing airway  
closed eyelids  
no response to any physical pressure.

MH:

patient have experienced severe chest pain prior to this  
and pain on left side of the body  
had uncontrolled bp  
shortness of breath and have been taking medications for ruptures in blood  
vessels .

objective :

vitals bp - 90/50

hr - 51

eyelids - responsive to light

but still unconscious

blood report - indicates presence of low level thrombocytes

rupture of blood vessels(aorta) found in mri scans

blood to brains have been limited and blood to other organs is lowered as well ( reported after taking into or)

Assessment :

Aortic embolism with rupture,

hypotension

ischemic conditions in multiple organs (higher chances )

possibility of brain dead (ruled out)

chances of more bleeding

chances of CHF.

Plan:

immediate treatment with anti hypotension drugs.

surgery required to control the rupture , while maintaining the bleeding

cbc monitoring to maintain thrombocyte level in body to control any bleeding.

supply of oxygenated blood in the system via ventilator.

preparation for post operational seizures, post operational rupture ,  
hypertension, bleeding.

reassurance to family.

ICD = I74.09 , I95.9 ,

CPT = 99215

SOAP #3

Name of patient - HK Kandeyathodiyil

age -55

sex male

location : patients residence

Subjective :

the patient has requested for audio/video telehealth service .

reason of call : the patient has reported severe pain in the abdomen region and  
has seen blood in vomit .

review of system

patient is responding well.

responding to every question asking

able to mention about the pain he has and about the severity of it

and also about the vomit

no other problems or symptoms visible.

slightly fatigued

MH:

the patient has been undergoing treatment for gastric ulcer and also been going  
through treatment for the perforation that has occurred

Objective :

vital : bp 117/94  
O2 level stable  
no dizziness or fatigue  
slight rise in temperature  
mri scan shows inflammation and lesions in the stomach lining caused due to ulcer  
and perforation has occurred once more in the stomach lining  
, hemorrhage is also present and blood in stomach lining causes the infection symptoms of patient.

Assessment:  
chronic ulcer with perforation and hemorrhage.  
gastritis  
fatigue

Plan:  
A surgery to treat the perforation.  
liquid form of diet until the perforation is completely treated and the bleeding is stopped  
no acid or heavy diet should be taken during this period  
anti-inflammatory drugs administered to treat the symptomatic gastritis and for the treatment of infection antibiotic is prescribed.  
Reassurance  
rest for recovery after the surgery  
no stress work for at least 1 week  
special care on diet to be taken to not start another lesion in the lining.  
  
if necessary removal of parts more vulnerable to inflammation.  
ICD = K25.6, K29.01  
CPT = 98007 - 95 modifier

SOAP #4  
name HARRY  
age 30  
gender male  
location - birla hospital, satna  
way - patient has opt for telehealth communication via audio only (apollo hospital, indore)

Subject  
cc : pt reported to have pain on the lower back side of the abdomen

mh:  
patient has been suffering extreme point pain in the region for quite a long time and has been checking up for the presence of kidney stone .  
patient has been advised to do regular ultrasound technique for removal of the stone but missed some sessions,  
and the size of the calculus has increased subsequently with time meaning the ultrasound technique is not effective .

review: the pt has been unable to sit properly feeling sharp pain in the back ,  
has reported blood in urine as well,  
not other physical symptoms are visible.

Objective : urine analysis : presence of infection indicators , calcium in urine , minerals as well  
urinalysis : increased level of wbc  
blood in urine has been confirmed  
crystals is also present in the blood

CT scans show blockages in the urinary tract  
x ray also confirmed the presence of renal calculi in the kidney region

Assessment :-

nephritis  
presence of renal calculi

plan:-

reassurance of faster recovery after stone removal  
diet requirements which need to be taken care of for better recovery  
water drinking for removal of any other stone fragments present in the urinary system  
regular ultrasound checkups to ensure no recurrence of the calculi  
treatment of the infection via anti microbial drug regimen  
rest for at least 2-3 weeks for faster recovery  
no stress /heavy lifting work do not risk any stitch opening resulting for any further complications  
final checkup to make sure the calculi is completely removed  
calcium food sources to be removed from diet to stop the possibility of calculi formation and liquid diet is to be ensured.

ICD = N08 , 50060

CPT = 98015-93modifier